SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Carboplatin + etoposide + chest radiotherapy

Stage III non-small-cell lung cancer when cisplatin is unsuitable · NCCP 00561 · version 5 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Chemotherapy and chest radiotherapy work together to treat lung cancer. Your team will explain the aim, possible benefit and alternatives for you.

Schedule
Carboplatin on Days 1 and 8; etoposide on Days 1–5. Each cycle lasts 28 days. Two cycles are planned. Radiotherapy usually starts within 24 hours of the first chemotherapy dose; follow your separate radiation appointment plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Nausea and vomiting
- ☐ Kidney and hearing problems
- ☐ Numbness or tingling
- ☐ Infusion reaction

Etoposide
CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Hair loss
- ☐ Sickness and mouth soreness
- ☐ Infusion reaction or low blood pressure
- ☐ Later treatment-related leukaemia

Chest radiotherapy
RADIOTHERAPY

- ☐ Tiredness
- ☐ Painful or difficult swallowing
- ☐ Sore skin in the treated area
- ☐ Cough or breathing changes during or after treatment

Discuss the aim and alternatives, blood-count suppression and serious infection, kidney/hearing/nerve injury, nausea, hair loss, infusion reactions, fertility, and chest radiation effects including swallowing and breathing problems. Etoposide for NSCLC is outside its licensed indication; explain the rationale and follow local off-label consent policy.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00561 v5.

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Developed by Dr Paul O'Brien